

EMETA 500mg Powder for Concentrate for Solution for Infusion

STRENGTH:

Each 50ml vial with 20mm rubber stopper and 20mm aluminum flip-off seal contains Pemetrexed Disodium equivalent to 500mg Pemetrexed.

PRODUCT DESCRIPTION:

EMETA 500mg Powder for Concentrate for Solution for Infusion is supplied as a sterile lyophilized powder for intravenous infusion available in single dose vials. This product is a white to either light yellow or green-yellow lyophilized solid. It is clear, colorless to yellow or green yellow solution after reconstituted with 0.9% Sodium Chloride. Each 50ml vial of Pemetrexed for Injection contains pemetrexed disodium equivalent to 500 mg pemetrexed.

PHARMACOLOGICAL PROPERTIES:

Pharmacodynamics properties

Pharmacotherapeutic group: Belongs to the class of folic acid analogues
ATC code: L01BA04.

Pemetrexed is a multi-targeted anti-cancer anti folate agent that exerts its action by disrupting crucial folate-dependent metabolic processes essential for cell replication. Pemetrexed behaves as a multi-targeted anti folate by inhibiting thymidylate synthase (TS), dihydrofolate reductase (DHFR), and glycinamide ribonucleotide formyltransferase (GARFT), which are key folate-dependent enzymes for the de novo biosynthesis of thymidine and purine nucleotides. Pemetrexed is transported into cells by both the reduced folate carrier and membrane folate binding protein transport systems. Once in the cell, pemetrexed is rapidly and efficiently converted to polyglutamate forms by the enzyme folylpolyglutamate synthetase. The polyglutamate forms are retained in cells and are even more potent inhibitors of TS and GARFT. Polyglutamation is a time-and concentration-dependent process that occurs in tumour cells and, to a lesser extent, in normal tissues. Polyglutamated metabolites have an increased intracellular half-life resulting in prolonged drug action in malignant cells.

Pharmacokinetic properties

The pharmacokinetic properties of pemetrexed following single-agent administration in cancer patients with a variety of solid tumours at doses ranging from 0.2 to 838mg/m² infused over a 10 minutes period shows a steady-state volume of distribution of 9 l/m². Pemetrexed is approximately 81% bound to plasma proteins. Binding was not notably affected by varying degrees of renal impairment. Pemetrexed undergoes limited hepatic metabolism. Pemetrexed is primarily eliminated in the urine, with 70% to 90% of the administered dose being recovered unchanged in urine within the first 24 hours following administration. Pemetrexed is actively secreted by OAT3 (organic anion transporter).

Pemetrexed total systemic clearance is 91.8 ml/min and the elimination half-life from plasma is 3.5 hours in patients with normal renal function (creatinine clearance of 90ml/min). Between-patient variability in clearance is moderate at 19.3%. Pemetrexed total systemic exposure

(AUC) and maximum plasma concentration increase proportionally with dose. The pharmacokinetics of pemetrexed are consistent over multiple treatment cycles.

The pharmacokinetic properties of pemetrexed is not influenced by concurrently administered cisplatin. Oral folic acid and intramuscular vitamin B12 supplementation do not affect the pharmacokinetics of pemetrexed.

INDICATIONS:

Malignant Pleural Mesothelioma:

EMETA in combination with cisplatin is indicated for the treatment of chemotherapy naive patients with unresectable malignant pleural mesothelioma.

Non-small cell lung cancer:

EMETA in combination with cisplatin is indicated for the first line treatment of patients with locally advanced or metastatic non-small cell lung cancer (NSCLC) other than predominantly squamous cell histology.

EMETA is indicated as monotherapy for the maintenance treatment of locally advanced or metastatic non-small cell lung cancer other than predominantly squamous cell histology in patients whose disease has not progressed immediately following platinum-based chemotherapy. EMETA is indicated as monotherapy for the second line treatment of patients with locally advanced or metastatic non-small cell lung cancer other than predominantly squamous cell histology.

DOSAGE AND ADMINISTRATION:

Posology

EMETA must only be administered under the supervision of a physician qualified in the use of anti-cancer chemotherapy.

EMETA in combination with cisplatin

The recommended dose of EMETA is 500 mg/m² of body surface area (BSA) administered as an intravenous infusion over 10 minutes on the first day of each 21-day cycle. The recommended dose of cisplatin is 75 mg/m² BSA infused over two hours approximately 30 minutes after completion of the pemetrexed infusion on the first day of each 21-day cycle. Patients must receive adequate anti-emetic treatment and appropriate hydration prior to and/or after receiving cisplatin (see also cisplatin product insert for specific dosing advice).

EMETA as single agent

In patients treated for non-small cell lung cancer after prior chemotherapy, the recommended dose of EMETA is 500 mg/m² BSA administered as an intravenous infusion over 10 minutes on the first day of each 21-day cycle.

Pre-medication regimen

To reduce the incidence and severity of skin reactions, a corticosteroid should be given the day prior to, on the day of, and the day after pemetrexed administration. The corticosteroid should be equivalent to 4 mg of dexamethasone administered orally twice a day (see section Warning & Precautions).

To reduce toxicity, patients treated with pemetrexed must also receive vitamin supplementation (see section Warning & Precautions). Patients must take oral folic acid or a multivitamin containing folic acid (350 to

1000 micrograms) on a daily basis. At least five doses of folic acid must be taken during the seven days preceding the first dose of pemetrexed, and dosing must continue during the full course of therapy and for 21 days after the last dose of pemetrexed. Patients must also receive an intramuscular injection of vitamin B12 (1000 micrograms) in the week preceding the first dose of pemetrexed and once every three cycles thereafter. Subsequent vitamin B12 injections may be given on the same day as pemetrexed.

Monitoring

Patients receiving pemetrexed should be monitored before each dose with a complete blood count, including a differential white cell count (WCC) and platelet count. Prior to each chemotherapy administration, blood chemistry tests should be collected to evaluate renal and hepatic function. Before the start of any cycle of chemotherapy, patients are required to have the following: absolute neutrophil count (ANC) should be ≥ 1500 cells/mm³ and platelets should be $\geq 100,000$ cells/mm³.

Creatinine clearance should be ≥ 45 ml/min.

The total bilirubin should be ≤ 1.5 times upper limit of normal. Alkaline phosphatase (AP), aspartate aminotransferase (AST or SGOT) and alanine aminotransferase (ALT or SGPT) should be ≤ 3 times upper limit of normal. Alkaline phosphatase, AST and ALT ≤ 5 times upper limit of normal is acceptable if liver has tumour involvement.

Dose adjustments

Dose adjustments at the start of a subsequent cycle should be based on nadir haematologic counts or maximum non-haematologic toxicity from the preceding cycle of therapy. Treatment may be delayed to allow sufficient time for recovery. Upon recovery patients should be retreated using the guidelines in Tables 1, 2 and 3, which are applicable for EMETA used as a single agent or in combination with cisplatin.

Table 1 - Dose modification table for EMETA (as single agent or in combination) and cisplatin-Haematologic toxicities	
Nadir ANC $<500/\text{mm}^3$ and nadir platelets $\geq 50,000/\text{mm}^3$.	75% of previous dose (both EMETA and cisplatin).
Nadir platelets $<50,000/\text{mm}^3$ without bleeding regardless of nadir ANC.	75% of previous dose (both EMETA and cisplatin).
Nadir platelets $<50,000/\text{mm}^3$ with bleeding ^a , regardless of nadir ANC.	50% of previous dose (both EMETA and cisplatin).

^aThese criteria meet the National Cancer Institute Common Toxicity Criteria (CTC v2.0; NCI 1998) definition of $\geq$ CTC Grade 2 bleeding

If patients develop non-haematologic toxicities $\geq$ Grade 3 (excluding neurotoxicity), EMETA should be withheld until resolution to less than or equal to the patient's pre-therapy value. Treatment should be resumed according to the guidelines in Table 2.

Table 2 - Dose modification table for EMETA (as single agent or in combination) and cisplatin- Non-haematologic toxicities ^{a, b}		
	Dose of EMETA (mg/m ²)	Dose of Cisplatin (mg/m ²)
Any Grade 3 or 4 toxicities except mucositis	75% of previous dose	75% of previous dose

Any diarrhea requiring hospitalization (irrespective of Grade) or Grade 3 or 4 diarrhea	75% of previous dose	75% of previous dose
Grade 3 or 4 mucositis	50% of previous dose	100% of previous dose

^aNational Cancer Institute Common Toxicity Criteria (CTC v2.0; NCI 1998)

^bExcluding neurotoxicity

In the event of neurotoxicity, the recommended dose adjustment for EMETA and cisplatin is documented in Table 3. Patients should discontinue therapy if Grade 3 or 4 neurotoxicity is observed.

Table 3 - Dose modification table for EMETA (as single agent or in combination) and cisplatin – Neurotoxicity		
CTC ^a Grade	Dose of EMETA (mg/m ²)	Dose of Cisplatin (mg/m ²)
0-1	100% of previous dose	100% of previous dose
2	100% of previous dose	50% of previous dose

^aNational Cancer Institute Common Toxicity Criteria (CTC v2.0; NCI 1998)

Treatment with EMETA should be discontinued if a patient experiences any haematologic or non-haematologic Grade 3 or 4 toxicity after 2 dose reductions or immediately if Grade 3 or 4 neurotoxicity is observed.

Elderly

In clinical studies, there has been no indication that patients 65 years of age or older are at increased risk of adverse events compared to patients younger than 65 years old. No dose reductions other than those recommended for all patients are necessary.

Paediatric population

There is no relevant use of EMETA in the paediatric population in malignant pleural mesothelioma and non-small cell lung cancer.

Patients with renal impairment (Standard Cockcroft and Gault formula or Glomerular Filtration Rate measured Tc99m-DPTA serum clearance method).

Pemetrexed is primarily eliminated unchanged by renal excretion. In clinical studies, patients with creatinine clearance of ≥ 45 ml/min required no dose adjustments other than those recommended for all patients. There are insufficient data on the use of pemetrexed in patients with creatinine clearance below 45 ml/min; therefore the use of pemetrexed is not recommended.

Patients with hepatic impairment

No relationships between AST (SGOT), ALT (SGPT), or total bilirubin and pemetrexed pharmacokinetics were identified. However patients with hepatic impairment such as bilirubin > 1.5 times the upper limit of normal and/or aminotransferase > 3.0 times the upper limit of normal (hepatic metastases absent) or > 5.0 times the upper limit of normal (hepatic metastases present) have not been specifically studied.

Method of administration

For Precautions to be taken before handling or administering EMETA, see section Instruction for Use

EMETA should be administered as an intravenous infusion over 10 minutes on the first day of each 21-day cycle. For instructions on reconstitution and dilution of EMETA before administration, see section Instruction for Use

CONTRAINDICATIONS:

- i. Hypersensitivity to the active substance or to any of the excipients
- ii. Breast-feeding
- iii. Concomitant yellow fever vaccine

PRECAUTIONS:

Pemetrexed can suppress bone marrow function as manifested by neutropenia, thrombocytopenia and anaemia (or pancytopenia) (see section Side Effects). Myelosuppression is usually the dose-limiting toxicity. Patients should be monitored for myelosuppression during therapy and pemetrexed should not be given to patients until absolute neutrophil count (ANC) returns to ≥ 1500 cells/mm³ and platelet count returns to $\geq 100,000$ cells/mm³. Dose reductions for subsequent cycles are based on nadir ANC, platelet count and maximum non-haematologic toxicity seen from the previous cycle (see section Recommended Dose).

Less toxicity and reduction in Grade 3/4 haematologic and non-haematologic toxicities such as neutropenia, febrile neutropenia and infection with Grade 3/4 neutropenia were reported when pre-treatment with folic acid and vitamin B12 was administered. Therefore, all patients treated with pemetrexed must be instructed to take folic acid and vitamin B12 as a prophylactic measure to reduce treatment-related toxicity (see section Recommended Dose).

Skin reactions have been reported in patients not pre-treated with a corticosteroid. Pre-treatment with dexamethasone (or equivalent) can reduce the incidence and severity of skin reactions (see section Recommended Dose).

An insufficient number of patients has been studied with creatinine clearance of below 45 ml/min. Therefore, the use of pemetrexed in patients with creatinine clearance of < 45 ml/min is not recommended (see section Recommended Dose).

Patients with mild to moderate renal insufficiency (creatinine clearance from 45 to 79 ml/min) should avoid taking non-steroidal anti-inflammatory drugs (NSAIDs) such as ibuprofen, and acetylsalicylic acid (> 1.3 g daily) for 2 days before, on the day of, and 2 days following pemetrexed administration (see section Drug Interaction).

In patients with mild to moderate renal insufficiency eligible for pemetrexed therapy NSAIDs with long elimination half-lives should be interrupted for at least 5 days prior to, on the day of, and at least 2 days following pemetrexed administration (see section Drug Interaction).

Serious renal events, including acute renal failure, have been reported with pemetrexed alone or in association with other chemotherapeutic agents. Many of the patients in whom these occurred had underlying risk factors for the development of renal events including dehydration or pre-existing hypertension or diabetes. Nephrogenic diabetes insipidus and renal tubular necrosis were also reported in post marketing setting with pemetrexed alone or with other chemotherapeutic agents. Most of these events resolved after pemetrexed withdrawal. Patients should be regularly monitored for acute tubular necrosis, decreased renal function and signs and symptoms of nephrogenic diabetes insipidus (e.g. hypernatraemia).

The effect of third space fluid, such as pleural effusion or ascites, on pemetrexed is not fully defined. Drainage of third space fluid collection prior to pemetrexed treatment should be considered, but may not be necessary.

Due to the gastrointestinal toxicity of pemetrexed given in combination with cisplatin, severe dehydration has been observed. Therefore, patients should receive adequate antiemetic treatment and appropriate hydration prior to and/or after receiving treatment.

Serious cardiovascular events, including myocardial infarction and cerebrovascular events have been uncommonly reported during clinical studies with pemetrexed, usually when given in combination with another cytotoxic agent. Most of the patients in whom these events have been observed had pre-existing cardiovascular risk factors (see section Side Effects).

Immunodepressed status is common in cancer patients. As a result, concomitant use of live attenuated vaccines is not recommended (see section Contraindications and section Drug Interactions).

Pemetrexed can have genetically damaging effects. Sexually mature males are advised not to father a child during the treatment and up to 6 months thereafter. Contraceptive measures or abstinence are recommended. Owing to the possibility of pemetrexed treatment causing irreversible infertility, men are advised to seek counselling on sperm storage before starting treatment.

Women of childbearing potential must use effective contraception during treatment with pemetrexed (see section Pregnancy and Lactation).

Cases of radiation pneumonitis have been reported in patients treated with radiation either prior, during or subsequent to their pemetrexed therapy. Particular attention should be paid to these patients and caution exercised with use of other radiosensitising agents.

Cases of radiation recall have been reported in patients who received radiotherapy weeks or years previously.

Excipients

EMETA 500 mg powder for concentrate for solution for infusion

This medicinal product contains approximately 51mg of sodium per vial. To be taken into consideration by patients on a controlled sodium diet.

Effects on ability to drive and use machines:

No studies on the effects on the ability to drive and use machines have been performed. However, it has been reported that pemetrexed may cause fatigue. Therefore, patients should be cautioned against driving or operating machines if this event occurs.

INTERACTIONS WITH OTHER MEDICAMENTS:

Pemetrexed is mainly eliminated unchanged renally by tubular secretion and to a lesser extent by glomerular filtration. Concomitant administration of nephrotoxic drugs (e.g. aminoglycoside, loop diuretics, platinum compounds, cyclosporin) could potentially result in delayed clearance of pemetrexed. This combination should be used with caution. If necessary, creatinine clearance should be closely monitored.

Concomitant administration of substances that are also tubularly secreted (e.g. probenecid, penicillin) could potentially result in delayed clearance of pemetrexed. Caution should be made when these drugs are combined with pemetrexed. If necessary, creatinine clearance should be closely monitored.

In patients with normal renal function (creatinine clearance ≥ 80 ml/min), high doses of non-steroidal anti-inflammatory drugs (NSAIDs, such as ibuprofen > 1600 mg/day) and acetylsalicylic acid at higher dose (≥ 1.3 g daily) may decrease pemetrexed elimination and, consequently,

increase the occurrence of pemetrexed adverse events. Therefore, caution should be made when administering higher doses of NSAIDs or acetylsalicylic acid, concurrently with pemetrexed to patients with normal function (creatinine clearance ≥ 80 ml/min).

In patients with mild to moderate renal insufficiency (creatinine clearance from 45 to 79 ml/min), the concomitant administration of pemetrexed with NSAIDs (e.g. ibuprofen) or acetylsalicylic acid at higher dose should be avoided for 2 days before, on the day of, and 2 days following pemetrexed administration (see section Warning and Precautions).

In the absence of data regarding potential interaction with NSAIDs having longer half-lives such as piroxicam or rofecoxib, the concomitant administration with pemetrexed in patients with mild to moderate renal insufficiency should be interrupted for at least 5 days prior to, on the day of, and at least 2 days following pemetrexed administration (see section Warning & Precautions). If concomitant administration of NSAIDs is necessary, patients should be monitored closely for toxicity, especially myelosuppression and gastrointestinal toxicity.

Pemetrexed undergoes limited hepatic metabolism. Results from in vitro studies with human liver microsomes indicated that pemetrexed would not be predicted to cause clinically significant inhibition of the metabolic clearance of drugs metabolised by CYP3A, CYP2D6, CYP2C9, and CYP1A2.

Interactions common to all cytotoxics

Due to the increased thrombotic risk in patients with cancer, the use of anticoagulation treatment is frequent. The high intra-individual variability of the coagulation status during diseases and the possibility of interaction between oral anticoagulants and anticancer chemotherapy require increased frequency of INR (International Normalised Ratio) monitoring, if it is decided to treat the patient with oral anticoagulants.

Concomitant use contraindicated: Yellow fever vaccine: risk of fatal generalised vaccinal disease (see section Contraindications).

Concomitant use not recommended: Live attenuated vaccines (except yellow fever, for which concomitant use is contraindicated): risk of systemic, possibly fatal, disease. The risk is increased in subjects who are already immunosuppressed by their underlying disease. Use an inactivated vaccine where it exists (poliomyelitis) (see section Warning and Precautions).

INCOMPATIBILITIES

Pemetrexed is physically incompatible with diluents containing calcium, including lactated Ringer's injection and Ringer's injection. In the absence of other compatibility studies this medicinal product must not be mixed with other medicinal products.

PREGNANCY AND LACTATION:

Contraception in males and females

Women of childbearing potential must use effective contraception during treatment with pemetrexed.

Pemetrexed can have genetically damaging effects. Sexually mature males are advised not to father a child during the treatment and up to 6 months thereafter. Contraceptive measures or abstinence are recommended.

Pregnancy

There are no data from the use of pemetrexed in pregnant women but pemetrexed, like other anti-metabolites, is suspected to cause serious birth defects when administered during pregnancy. Animal studies have shown reproductive toxicity (see section 5.3). Pemetrexed should not be used during pregnancy unless clearly necessary, after a careful

consideration of the needs of the mother and the risk for the foetus (see section Warning and Precautions).

Breast-feeding

It is not known whether pemetrexed is excreted in human milk and adverse reactions on the suckling child cannot be excluded. Breast-feeding must be discontinued during pemetrexed therapy (see section Contraindications).

Fertility

Owing to the possibility of pemetrexed treatment causing irreversible infertility, men are advised to seek counselling on sperm storage before starting treatment.

SIDE EFFECTS:

Summary of the safety profile

The most commonly reported undesirable effects related to pemetrexed, whether used as monotherapy or in combination, are bone marrow suppression manifested as anaemia, neutropenia, leukopenia, thrombocytopenia; and gastrointestinal toxicities, manifested as anorexia, nausea, vomiting, diarrhoea, constipation, pharyngitis, mucositis, and stomatitis. Other undesirable effects include renal toxicities, increased aminotransferases, alopecia, fatigue, dehydration, rash, infection/sepsis and neuropathy. Rarely seen events include Stevens-Johnson syndrome and Toxic epidermal necrolysis.

Tabulated list of Adverse Reactions

Emeta in Combination with Cisplatin (Mesothelioma Patients)

System Organ Class	Frequency	Event*
Blood and lymphatic system disorders	Very common	Neutrophils/ Granulocytes decreased
		Leukocytes decreased
		Haemoglobin decreased
		Platelets decreased
Metabolism and nutrition disorders	Common	Dehydration
Nervous system disorders	Very common	Neuropathy- Sensory
	Common	Taste disturbance
Eye disorder	Common	Conjunctivitis
Gastrointestinal disorders	Very common	Diarrhoea
		Vomiting
		Stomatitis/Pharyngitis
		Nausea
		Anorexia
		Constipation
	Common	Dyspepsia
Skin and subcutaneous tissue disorders	Very common	Rash
		Alopecia

Renal and urinary disorders	Very common	Creatinine elevation
		Creatinine clearance decreased**
General disorders and administration site conditions	Very common	Fatigue

*Refer to National Cancer Institute CTC version 2 for each grade of toxicity except the term “creatinine clearance decreased”

**which is derived from the term “renal/genitourinary other”.

*** According to National Cancer Institute CTC (v2.0; NCI 1998), taste disturbance and alopecia should only be reported as Grade 1 or 2

Emeta (single agent) in with locally advanced or metastatic NSCLC Patients and had received prior chemotherapy

System Organ Class	Frequency	Event***
Blood and lymphatic system disorders	Very Common	Neutrophils/ Granulocytes decreased
		Leukocytes decreased
		Haemoglobin decreased
	Common	Platelets decreased
Gastrointestinal disorders	Very common	Diarrhoea
		Vomiting
		Stomatitis/Pharyngitis
		Nausea
		Anorexia
	Common	Constipation
Hepatobiliary Disorder	Common	SGPT (ALT) Elevation
		SGOT (ALT) Elevation
Skin and subcutaneous tissue disorders	Very common	Rash/desquamation
	Common	Pruritus
		<u>Alopecia</u>
General disorders and administration site conditions	Very common	Fatigue
	Common	Fever

***According to National Cancer Institute CTC (v2.0; NCI 1998), alopecia should only be reported as Grade 1 or 2.

Emeta in Combination with Cisplatin initial treatment for locally advanced or metastatic NSCLC

System Organ Class	Frequency	Event**
Blood and lymphatic system disorders	Very common	Neutrophils/ Granulocytes decreased
		Leukocytes decreased

		Haemoglobin decreased
		Platelets decreased
Nervous system disorders	Common	Neuropathy- Sensory
		Taste disturbance
Gastrointestinal disorders	Very common	Nausea
		Vomiting
		Anorexia
		Constipation
		Stomatitis/Pharyngitis
		Diarrhea without colostomy
	Common	Dyspepsia/heartburn
Skin and subcutaneous tissue disorders	Very common	Alopecia
	Common	Rash/desquamation
Renal and urinary disorders	Very common	Creatinine elevation
General disorders and administration site conditions	Very common	Fatigue

**Refer to National Cancer Institute CTC (v2.0; NCI 1998) for each Grade of Toxicity.

Emeta (single agent) In Stage IIIB or IV NSCLC patients and had received prior platinum-based chemotherapy

System Organ Class	Frequency	Event**
Blood and lymphatic system disorders	Very common	Hemoglobin decreased
	Common	Leukocytes decreased
		Neutrophils decreased
Nervous system disorders	Common	Neuropathy- Sensory
Gastrointestinal disorders	Very common	Nausea
		Anorexia
	Common	Vomiting
		Mucositis/Stomatitis
Hepatobiliary disorders	Common	ALT (SGPT) elevation
		AST (SGOT) elevation
Skin and subcutaneous tissue disorders	Common	Rash/desquamation
General disorders and administration site conditions	Very common	Fatigue
	Common	Pain
		Edema
Renal Disorders	Common	Renal disorders****

****Refer to NCI CTCAE Criteria (Version 3.0; NCI 2003) for each grade of toxicity. The reporting rates shown are according to CTCAE version 3.0.**

******** Combined term includes increased serum/blood creatinine, decreased glomerular filtration rate, renal failure and renal/genitourinary- other.

Toxicities with respect to gender were similar to the overall population in patients receiving pemetrexed plus cisplatin.

Serious cardiovascular and cerebrovascular events, including myocardial infarction, angina pectoris, cerebrovascular accident and transient ischaemic attack have been uncommonly reported during clinical studies with pemetrexed, usually when given in combination with another cytotoxic agent. Most of the patients in whom these events have been observed had pre-existing cardiovascular risk factors.

Rare cases of hepatitis, potentially serious, have been reported during clinical studies with pemetrexed.

Pancytopenia has been uncommonly reported during clinical trials with pemetrexed.

In clinical trials, cases of colitis (including intestinal and rectal bleeding, sometimes fatal, intestinal perforation, intestinal necrosis and typhlitis) have been reported uncommonly in patients treated with pemetrexed.

In clinical trials, cases of interstitial pneumonitis with respiratory insufficiency, sometimes fatal, have been reported uncommonly in patients treated with pemetrexed.

Uncommon cases of oedema have been reported in patients treated with pemetrexed.

Oesophagitis/ radiation oesophagitis has been uncommonly reported during clinical trials with pemetrexed.

Sepsis, sometimes fatal, has been commonly reported during clinical trials with pemetrexed.

During post marketing surveillance, the following adverse reactions have been reported in patients treated with pemetrexed:

Hyperpigmentation has been commonly reported.

Uncommon cases of acute renal failure have been reported with pemetrexed alone or in association with other chemotherapeutic agents . Nephrogenic diabetes insipidus and renal tubular necrosis have been reported in post marketing setting with an unknown frequency.

Uncommon cases of radiation pneumonitis have been reported in patients treated with radiation either prior, during or subsequent to their pemetrexed therapy.

Rare cases of radiation recall have been reported in patients who have received radiotherapy previously.

Uncommon cases of peripheral ischemia leading sometimes to extremity necrosis have been reported.

Rare cases of bullous conditions have been reported including Stevens-Johnson syndrome and Toxic epidermal necrolysis which in some cases were fatal.

Rarely, immune-mediated haemolytic anemia has been reported in patients treated with pemetrexed.

Rare cases of anaphylactic shock have been reported.

Erythematous oedema mainly of the lower limbs has been reported with an unknown frequency. Infectious and non-infectious disorders of the dermis, the hypodermis and/or the subcutaneous tissue have been reported with an unknown frequency (e.g. acute bacterial dermo-hypodermatitis, pseudocellulitis, dermatitis).

OVERDOSAGE:

Reported symptoms of overdose include neutropenia, anaemia, thrombocytopenia, mucositis, sensory polyneuropathy and rash. Anticipated complications of overdose include bone marrow suppression as manifested by neutropenia, thrombocytopenia and anaemia. In addition, infection with or without fever, diarrhoea, and/or mucositis may be seen. In the event of suspected overdose, patients should be monitored with blood counts and should receive supportive therapy as necessary. The use of calcium folinate / folinic acid in the management of pemetrexed overdose should be considered

LIST OF EXCIPIENTS:

Each vial contains:

Mannitol
Water for injection
Sodium Hydroxide
Hydrochloric Acid

PACKAGING

EMETA 500mg Powder for Concentrate for Solution for Infusion in Type 1 clear vial with 20mm Chlorobutyl rubber stopper and 20mm aluminum flip-off seal. 1 vial was packed into one printed box and wadding was put in for one vial in one box.

STORAGE CONDITION:

Unopened vial

Store below 30 °C. Protect from light and moisture.

Reconstituted and infusion solutions

Reconstituted and infusion solutions: Store reconstituted product under refrigerated conditions between 2°C to 8°C for no longer than 24 hours from the time of reconstitution. Discard vial after 24 hours.

Keep out of reach of children.

SHELF LIFE:

Unopened vial

2years. Protect from light and moisture.

Reconstituted and infusion solutions

Reconstituted and infusion solutions: Store reconstituted product under refrigerated conditions between 2°C to 8°C for no longer than 24 hours from the time of reconstitution. Discard vial after 24 hours.

Keep out of reach of children.

Product Registration Holder

Nano Medic Care Sdn Bhd

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